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### Author reply

Dear Editor:
We read Dr. Eisenberg's letter with the utmost care. As far as the items needing clarifications, we can offer the following considerations:

1. The aim of our study was to verify if patients who did not respond to latanoprost were responders to bimatoprost. The observed results offer a clear answer to this question. Discussing the biochemistry of bimatoprost is far beyond the aims of our study. However, while discussing our results, we carefully pointed out that both (1) poor esterification of the prodrug latanoprost to the active acid form and (2) differences in the involved receptors might offer an explanation for the observed phenomenon. Moreover, the characterization of bimatoprost as a prostamide was suggested by Woodward et al in 2001.[1] Another article produced by the same authors and offering more details has recently been published.[2]
2. We were quoting the results of a multicenter, prospective, randomized clinical trial that appeared in a peer-reviewed journal.[3] This procedure is, in our opinion, appropriate and correct and follows the rules of evidence-based medicine. We are well aware of the post hoc criticisms raised by Drs. Eisenberg, Toris, and Camras in a review published, under the sponsorship of Pharmacia, in a supplement of *Survey of Ophthalmology*.[4] The issue of the face-to-face comparison between latanoprost and bimatoprost is still a matter of debate[5,6] and is not pertinent to our nonresponders study.
3. Dr. Eisenberg states that "it is difficult to generalize the findings of this study to clinical practice" and "clinically there are patients who respond to latanoprost but do not respond to bimatoprost," and asks "What would a study of this group prove?" To the best of our knowledge, our study is the first one to give proper scientific support to the general anecdotal experience quoted by Dr. Eisenberg (i.e., "everyone who practices medicine encounters patients who respond in unusual ways"). In our opinion, this is the most correct way to move from evidence-b(i)ased medicine to evidence-based medicine. Finally, the big family of the so-called hypotensive lipids (i.e., travoprost, latanoprost, bimatoprost, unoprostone) may be rather unique in the heterogeneity of the response given by each individual patient to each individual drug: does it make sense, whenever a patient gives a suboptimal response to a member of this family, to shift to another member of the same family instead of shifting to another family of drugs? Our controlled clinical trial gives some scientific support to this hypothesis.

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## MRI to Differentiate Causes of Optic Nerve Disease

Dear Editor:
Rizzo et al[1] reported a retrospective evaluation of the use of magnetic resonance imaging (MRI) to differentiate optic neuritis from nonarteritic anterior ischemic optic neuropathy (NAION). They found that 5 of 32 patients with NAION had abnormally short $T_1$ inversion recovery signals on MRI in the clinically affected eye and that 2 patients had enhancement of the optic nerve. We have previously reported optic nerve enhancement on MRI in arteritic ischemic optic neuropathy.[2] Although it is not customary to perform an MRI for NAION, many patients arrive in the neuroophthalmology clinic with the outside MRI in hand. It has been my clinical impression that NAION does not produce enhancement of the optic nerve. When I have seen enhancement of the optic nerve in patients with a clinical picture suggestive of NAION I have pursued a more aggressive evaluation for inflammatory optic neuropathy (e.g., sarcoid) and giant cell arteritis (i.e., temporal artery biopsy).

It has been my assumption that small-vessel central nervous system ischemia (e.g., the presumed ischemic subcortical and periventricular white matter lesions) does not produce enhancement but that demyelination (e.g., acute periventricular white matter lesions in multiple sclerosis) and vasculitis (e.g., giant cell arteritis) do produce enhancement. I wonder if the authors could comment on (1) the possibility of giant cell arteritis in the two cases they reported of presumed NAION and optic nerve enhancement and (2) the mechanism for the presence (or absence) of optic nerve enhancement in NAION.

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